SUPPLEMENTARY DATA

Clinical outcomes of very short term dual antiplatelet therapy in patients with or without diabetes undergoing second-generation drug-eluting stents: a systematic review and meta-analysis of randomized clinical trials

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Supplementary Table 1. Outcome definitions of the randomized controlled trials included.

Study	NACE	MACCE	cardiac death	MI	Stroke	ST	Bleeding
TICO	MACCE plus major bleeding as defined by TIMI.	Composite outcome of all-cause death, MI, ST, stroke, and TVR.	death due to MI, cardiac perforation or pericardial tamponade, an arrhythmia or conduction abnormality, stroke within 30 days of the procedure or related to the procedure, death due to a procedural complication.	Third universal definition	An acute cerebrovascular event resulting in death or neurological deficit＞ 24 hours or the presence of acute infarction demonstrated by imaging studies.	ARC criteria	TIMI criteria.
TWILIGHT	-	-	Any death due to proximate cardiac cause, unwitnessed death and death of unknown cause, all procedure-related deaths including those related to concomitant treatment.	Third universal definition	acute symptomatic episode of neurological dysfunction, more than 24 hours in duration in the absence of therapeutic intervention or death, due to cerebral, spinal or retinal tissue injury as evidenced by neuroimaging or lumbar puncture.	ARC criteria	BARC criteria.

STOPDAPT-2	cardiovascular death, MI, definite ST stroke, or TIMI major or minor bleeding.	cardiovascular end point (a composite of cardiovascular death, MI, definite ST, or stroke.	-	ARC criteria.		ARC criteria	TIIMI criteria.
SMART-CHOICE	-	-	All deaths were considered cardiac unless a definite non-cardiac cause could be established.	Elevated cardiac enzyme levels above the URL with ischemic symptoms or ECG findings indicative of ischemia.	Non- convulsive focal or global neurologic deficit of abrupt onset lasting for more than 24 hours or leading to death, which was caused by ischemia or hemorrhage within the brain.	ARC criteria	BARC criteria.
REDUCE	MACCE plus BARC 2 to 5 bleeding.	-	-	Third universal definition.	-	ARC criteria.	BARC criteria.

GLOBAL-LEADERS	-	-	-	Third universal definition.	-		BARC criteria.
OPTIMIZE	-	-	Any death unless a definite non-cardiovascular cause could be established.	Per-procedural: rise in cardiac enzyme within 48hs after PCI or >5 times ULN, with new Q wave/LBBB after CABG.	Acute neurological event with duration ⩾24hs with confirmation by either CT or MRI or pathological confirmation.	ARC criteria.	REPLACE criteria.
RESET	composite of cardiovascular death, MI, ST, TVR, or bleeding.	-	Any death unless a definite non-cardiovascular cause could be established.	Typical symptoms, with EKG changes and a rise in cardiac enzyme*, unrelated to an interventional procedure.	Sudden onset of vertigo, numbness, aphasia, or dysarthria resulting from vascular lesions of the brain, including hemorrhage, embolism, thrombosis, or rupturing aneurysm.	ARC criteria.	TIIMI criteria.

Abbreviations: ST= stent thrombosis , ARC=Academic Research Consortium; BARC=Bleeding Academic Research Consortium; CABG=coronary artery bypass grafting; CPK-MB= creatine phosphokinase myocardial band; CT=computer tomography; EKG=electrocardiography; LBB=left bundle branch block; MRI= magnetic resonance imaging; PCI=percutaneous coronary intervention; REPLACE=Randomized Evaluation of PCI Linking Angiomax to Reduced Clinical Events; TIMI=thrombolysis in myocardial infarction; TVR=target vessel revascularization; ULN=upper limit of normal.
*levels CPK-MB >3 times the ULN range or troponin T/I > than the 99th percentile of the ULN

Supplementary Table 2. Inclusion, exclusion criteria and primary outcomes of studies included.

Study	Major inclusion criteria	Major exclusion criteria	Primary outcomes	Primary Outcomes Results
TICO	ACS implanted bioresorbable polymer sirolimus-eluting stent.	Increased risk of bleeding, need for oral anticoagulation therapy, currently treated with strong CYP3A4 inhibitors.	composite of major bleeding (TIMI) and adverse cardiac and cerebrovascular events (death, MI, ST, stroke, and TVR).	superiority demonstrated.
TWILIGHT	High-risk patients who have undergone successful PCI with at least 1 locally approved drug-eluting stent discharged on DAPT with aspirin and ticagrelor for at least 3 month.	Need for oral anticoagulants; History of intracranial hemorrhage.	Bleeding Academic Research Consortium (BARC) 2, 3 or 5 bleeding.	Non-inferiority demonstrated.
STOPDAPT-2	Patients who have undergone PCI with the everolimus-eluting cobalt-chromium stent and have not experienced major complications during hospital stay for treatment.	requiring oral anticoagulants, history of intracranial hemorrhage, experienced major complications during hospital stay post-PCI.	composite of cardiovascular death, MI, ischemic or hemorrhagic stroke, definite ST, or major or minor bleeding.	Non-inferiority and superiority demonstrated.
SMART-CHOICE	stable ischemic or ACS with more than 50% diameter stenosis and a visually estimated diameter of ≥2.25. mm and ≤4.25 mm	cardiogenic shock, active pathologic bleeding, history of DES implantation within 12 months.	composite of all-cause death, MI, or stroke.	Non-inferiority demonstrated.

REDUCE	ACS patients successfully treated with COMBO stent.	cardiogenic shock, recent major bleeding complications or contraindication to DAPT.	composite of all-cause mortality, MI, ST, stroke, TVR and bleeding.	Non-inferiority demonstrated.
GLOBAL-LEADERS	any clinical indication for percutaneous coronary intervention, with more than 50% diameter stenosis.	fibrinolytic therapy within 24 hours of PCI, scheduled elective surgery within 12 months, previous stent thrombosis, stroke within last 30 days.	all-cause death or new Q-wave MI.	superiority not demonstrated.
OPTIMIZE	Stable angina or low risk unstable angina with at least 1 lesion in native coronary vessel ⩾2.5 mm in diameter.	STEMI, scheduled elective surgery within 12 months, in stent restenosis of DES, BMS in non-target vessel in the last 6 months.	Death, MI, CVA(stroke) or major bleeding.	Non-inferiority demonstrated.
RESET	Stable, unstable angina or acute MI with more than 50% diameter stenosis in a coronary artery.	Cardiogenic shock, STEMI within 48 hours, LVEF<40%, previous stent thrombosis, CTO.	Cardiac death, MI, ST, TVR or major bleeding.	Non-inferiority demonstrated.

Abbreviations: ACS=acute coronary syndrome; CYP3A4=cytochrome P450 3A4; DAPT=dual antiplatelet therapy; TIMI= thrombolysis in myocardial infarction; MI=myocardial infarction; ST=stent thrombosis; TVR=target vascular revascularization; STEMI=ST-segment elevated myocardial infarction; PCI=percutaneous coronary intervention; DES=drug eluting stents; CTO=chronic total occlusion; BARC=Bleeding Academic Research Consortium.

Supplementary Table 3. Clinical, angiographic and procedural characteristics of patients with or without diabetes.

Study	TWILIGHT		GLOBEL-LEADERS		*P*
	Diabetes (N=2620)	Non-diabetes (N=4499)	Diabetes (N=4038)	Non-diabetes (N=11919)	
Age, years mean ± SD	64.8±10.1	65.4±10.1	66.33±9.48	63.93±10.5	<0.0001
Female	23.6	24.0	26.4	22.18	<0.0001
BMI, kg/m^2, mean ± SD	29.8±6.0	27.9±5.2	29.66±5.01	27.69±4.33	<0.0001
chronic kidney disease	21.1	13.6	20.81	11.25	<0.0001
Anemia	25.6	15	21.31	10.76	<0.0001
Current Smoker	18.1	23,9	19.81	28.22	<0.0001
Hypercholesterolemia	66.6	56.9	78.38	66.65	<0.0001
Hypertension	81.5	67.1	86.69	69.16	<0.0001
Peripheral arterial disease	8.4	6.0	10.04	5.11	<0.0001
Previous MI	29.3	28.3	27.0	22.06	<0.0001
Previous PCI	45.5	40.1	40.22	30.2	<0.0001
Previous CABG	12.6	8.4	9.25	4.97	<0.0001
Previous major bleeding	0.9	0.9	0.6	0.62	0.79
Clinical presentation					0.0036
Stable coronary artery disease	1006(38.4)	509(33.3)	2434(60.28)	6041(50.68)	
Acute coronary syndrome	1614(61.6)	810(66.7)	1604(39.72)	5878(49.32)	
Target Vessel					
Left Main	4.7	5.1	2.92	2.64	0.25
LAD	55.5	56.7	48.98	51.47	0.07
LCX	33.9	31.3	33.47	30.99	<0.0001
RCA	35	35.2	36.81	37.84	0.16
Number of lesion treated, mean ± SD	1.5±0.8	—	1.46±0.75	1.43±0.74	0.05

Total stent length(mm), mean ± SD	39.5±24.0	40.1±24.4	36.21±25.54	35.24±25.14	0.01
Minimum stent diameter(mm), mean ± SD	2.8±0.5	2.9±0.5	—	—	＜0.0001
Stent number per patient	—	—	1.75±1.09	1.71±1.08	＜0.0001
Bifurcation PCI	11.6	12.5	14.74	16.1	0.02
Multi-vessel CAD/PCI	68.6	59.3	23.23	22.36	0.11
Average stent diameter per patient (mm), mean ± SD	—	—	2.96±0.43	3.01±0.43	＜0.0001

Abbreviations: LAD= left anterior descending, LCX= left circumflex artery, RCA= Right coronary artery. PCI=percutaneous coronary intervention. MI=myocardial infarction, CABG= Coronary Artery Bypass Grafting. CAD=coronary artery disease. Values shown are % (n) otherwise indicated.

Supplementary Table 4. Summary of GRADE evidence quality evaluation.

Patient or population: patients with diabetes following percutaneous coronary intervention.
Settings:
Intervention: De-escalation dual antiplatelet therapy.
Comparison: standard dual antiplatelet therapy.

Outcomes	**Illustrative comparative risks* (95% CI)**		**Relative effect (95% CI)**	**No of Participants (studies)**	**Quality of the evidence (GRADE)**	**Comments**
	Assumed risk	Corresponding risk				
	Standard dual antiplatelet therapy	**De-escalation dual antiplatelet therapy**				
net adverse clinical events Follow-up: median 12 months	**Study population**		**RR 0.72** (0.6 to 0.88)	6311 (6 studies)	⊕⊕⊕⊖ **moderate**[1]	
	74 per 1000	**53 per 1000** (44 to 65)				
	Low					
	44 per 1000	**31 per 1000** (26 to 38)				
	High					
	104 per 1000	**75 per 1000** (62 to 92)				
major bleeding Follow-up: median 12 months	**Study population**		**RR 0.69** (0.35 to 1.36)	8596 (4 studies)	⊕⊕⊕⊖ **moderate**[1]	
	25 per 1000	**17 per 1000** (9 to 34)				
	Low					

	High				
	43 per 1000	**30 per 1000** (15 to 58)			
major or minor bleeding	**Study population**		**RR 0.69**	3742	⊕⊕⊕⊖
Follow-up: median 12 months	**55 per 1000**	**38 per 1000** (29 to 51)	(0.52 to 0.93)	(2 studies)	**moderate**[1]
	Low				
	29 per 1000	**20 per 1000** (15 to 27)			
	High				
	66 per 1000	**46 per 1000** (34 to 61)			
major cardiac or cerebrovascular events	**Study population**		**RR 0.87**	8616	⊕⊕⊕⊖
Follow-up: median 12 months	**114 per 1000**	**99 per 1000** (88 to 112)	(0.77 to 0.98)	(4 studies)	**moderate**[1]
	Low				
	36 per 1000	**31 per 1000** (28 to 35)			
	High				
	186 per 1000	**162 per 1000** (143 to 182)			
all-cause death	**Study population**		**RR 0.82**	7707	⊕⊕⊕⊖
Follow-up: median 12 months	**37 per 1000**	**30 per 1000**	(0.64 to	(3 studies)	**moderate**[1]

		(23 to 38)	1.04)		
	Low				
	20 per 1000	**16 per 1000** (13 to 21)			
	High				
	48 per 1000	**39 per 1000** (31 to 50)			
cardiac death Follow-up: median 12 months	**Study population**		**RR 0.77** (0.56 to 1.06)	7999 (4 studies)	⊕⊕⊕⊖ **moderate**[1]
	22 per 1000	**17 per 1000** (12 to 23)			
	Low				
	7 per 1000	**5 per 1000** (4 to 7)			
	High				
	27 per 1000	**21 per 1000** (15 to 29)			
myocardial infarction Follow-up: median 12 months	**Study population**		**RR 0.93** (0.75 to 1.17)	7999 (4 studies)	⊕⊕⊕⊖ **moderate**[1]
	39 per 1000	**36 per 1000** (29 to 45)			
	Low				
	7 per 1000	**6 per 1000** (5 to 8)			
	High				

	41 per 1000	**38 per 1000** (31 to 48)			
stent thrombosis	**Study population**		**RR 0.92**	9158	⊕⊕⊕⊖
Follow-up: median 12 months	**8 per 1000**	**7 per 1000** (5 to 12)	(0.58 to 1.47)	(5 studies)	**moderate**[1]
	Low				
	2 per 1000	**2 per 1000** (1 to 2)			
	High				
	11 per 1000	**10 per 1000** (6 to 16)			
stroke	**Study population**		**RR 0.97**	7707	⊕⊕⊕⊖
Follow-up: median 12 months	**11 per 1000**	**10 per 1000** (7 to 16)	(0.63 to 1.5)	(3 studies)	**moderate**[1]
	Low				
	4 per 1000	**3 per 1000** (2 to 5)			
	High				
	17 per 1000	**16 per 1000** (10 to 25)			

*The basis for the **assumed risk** (e.g. the median control group risk across studies) is provided in footnotes. The **corresponding risk** (and its 95% confidence interval) is based on the assumed risk in the comparison group and the **relative effect** of the intervention (and its 95% CI).
CI: Confidence interval; **RR:** Risk ratio;

GRADE Working Group grades of evidence

High quality: Further research is very unlikely to change our confidence in the estimate of effect.

Moderate quality: Further research is likely to have an important impact on our confidence in the estimate of effect and may change the estimate.

Low quality: Further research is very likely to have an important impact on our confidence in the estimate of effect and is likely to change the estimate.

Very low quality: We are very uncertain about the estimate.

[1] Most of included trials were open label.

Supplementary Table 5. Subgroup analysis of diabetes management strategy.

	insulin dependent				non-insulin dependent					
	Total (n=1932)	1-3 months (n=940)	12 months (n=991)	P^a	Total (n=4710）	1-3 months (n=2408)	12 months (n=2282)	P^b	RR [95% CI]	P^c
Bleeding outcome										
major bleeding	54 (2.80%)	28 (2.98%)	26 (2.62%)	0.94	99 (2.10%)	38 (1.86%)	61 (2.67%)	0.26	1.32 [0.95, 1.83]	0.1
Ischemic outcomes										
all-cause death	90 (4.66%)	37 (3.94%)	53 (5.35%)	0.12	124 (2.63%)	30 (1.46%)	32 (1.40%)	0.31	1.72 [1.32, 2.24]	＜0.0001
cardiac death	49 (2.54%)	37 (3.94%)	50 (2.62%)	0.14	72 (1.53%)	15 (0.73%)	19 (0.83%)	0.42	1.63 [1.14, 2.34]	0.008
myocardial infarction	95 (4.92%)	51 (5.43%)	44 (2.62%)	0.57	168 (3.57%)	79 (3.86%)	89 (3.90%)	0.26	1.43 [0.94, 2.18]	0.1
ischemic stroke	34 (1.76%)	17 (1.81%)	14 (2.62%)	0.52	45 (0.96%)	19 (0.93%)	19 (0.83%)	0.85	1.87 [1.05, 3.31]	0.03
definite or probable ST	14 (0.72%)	6 (0.64%)	8 (2.62%)	0.69	45 (0.96%)	23 (1.12%)	22 (0.96%)	0.98	0.75 [0.41, 1.36]	0.34

The data was summarized from trials of GLOBAL-LEADERS and TWILIGHT. [a] *P* value for insulin dependent diabetes patients according to duration of DAPT, [b] *P* value for non-insulin dependent diabetes patients according to duration of DAPT, [c] *P* value for outcomes dependent on management of diabetes. ST=stent thrombosis.

Supplementary Table 6. Subgroup analysis based on clinical manifestation.

		1-3 months	12 months	RR [95% CI]	*P*
Total					
NACE	ACS	119/2255	150/2262	0.81 [0.55, 1.19]	0.28
	any CAD	333/7777	389/7787	0.86 [0.73, 1.01]	0.06
major bleeding	ACS	25/1527	45/1529	0.56 [0.34, 0.90]	0.02
	any CAD	202/13092	244/13103	0.73 [0.42, 1.25]	0.25
MACCE	ACS	35/1527	51/1529	0.69 [0.45, 1.05]	0.08
	any CAD	1226/13021	1303/13046	0.94 [0.87, 1.01]	0.1
Diabetes mellitus					
NACE	ACS	44/576	51/562	0.84 [0.57, 1.24]	0.38
	any CAD	126/2604	181/2507	0.69 [0.55, 0.86]	0.0009
major bleeding	ACS	12/418	18/417	0.67 [0.32, 1.36]	0.27
	any CAD	67/3922	87/3839	0.71 [0.26, 2.00]	0.52
MACCE	ACS	14/418	21/417	0.67 [0.34, 1.29]	0.23
	any CAD	420/3938	464/3843	0.88 [0.78, 1.00]	0.04
Non- diabetes mellitus					
NACE	ACS	75/1679	99/1701	0.77 [0.52, 1.16]	0.21
	any CAD	207/5173	208/5217	0.98 [0.77, 1.24]	0.87
major bleeding	ACS	13/1109	27/1112	0.48 [0.25, 0.93]	0.03
	any CAD	202/13092	244/13103	0.73 [0.42, 1.25]	0.25
MACCE	ACS	21/1109	30/1112	0.70 [0.40, 1.22]	0.21
	any CAD	806/9083	839/9203	1.03 [0.85, 1.24]	0.79

Abbreviations: ACS=acute coronary syndrome, CAD=coronary artery disease, NACE=net adverse clinical event, MACCE= major cardiac or cerebrovascular events.

Supplementary Table 7. Subgroup analysis of $P2Y_{12}$ or aspirin monotherapy after very short term DAPT.

Total		1-3 months	12 months	RR [95% CI]	*P*
NACE	$P2Y_{12}$	257/6582	340/6602	0.76 [0.65, 0.89]	0.0006
	aspirin	195/3450	199/3447	0.98 [0.81, 1.19]	0.83
all-cause death	$P2Y_{12}$	258/11498	297/11471	0.87 [0.73, 1.02]	0.09
	aspirin	43/1563	45/1556	0.95 [0.63, 1.44]	0.81
cardiac death	$P2Y_{12}$	127/11498	147/11471	0.86 [0.68, 1.09]	0.22
	aspirin	31/2622	36/2614	0.86 [0.53, 1.38	0.53
MI	$P2Y_{12}$	343/11498	345/11471	0.99 [0.86, 1.15]	0.91
	aspirin	51/2622	46/2614	1.11 [0.71, 1.64]	0.62
ST	$P2Y_{12}$	100/12998	102/12980	0.98 [0.74, 1.29]	0.88
	aspirin	9/2622	7/2614	1.28 [0.48, 3.44]	0.62
stroke	$P2Y_{12}$	104/11498	111/11471	0.93 [0.72, 1.22]	0.62
	aspirin	5/1553	5/1556	1.00 [0.29, 3.43]	0.99
major bleeding	$P2Y_{12}$	222/13056	283/13076	0.79 [0.66, 0,94]	0.007
	aspirin	5/1563	6/1556	0.83 [0.25, 2.71]	0.76
Diabetes mellitus					
NACE	$P2Y_{12}$	114/2322	173/2292	0.65 [0.52, 0.82]	0.0002
	aspirin	56/858	59/839	0.92 [0.65, 1.32]	0.66
all-cause death	$P2Y_{12}$	96/3357	120/3247	0.78 [0.60, 1.01]	0.06
	aspirin	21/554	19/549	1.10 [0.60, 2.01]	0.77
cardiac death	$P2Y_{12}$	52/3357	70/3247	0.72 [0.50, 1.03]	0.07
	aspirin	16/700	16/695	0.99 [0.50, 1.96]	0.98
MI	$P2Y_{12}$	130/3357	134/3247	0.91 [0.64, 1.30]	0.61
	aspirin	17/700	19/695	0.90 [0.47, 1.70]	0.74
ST	$P2Y_{12}$	30/3942	32/3821	0.91 [0.55, 1.50]	0.71

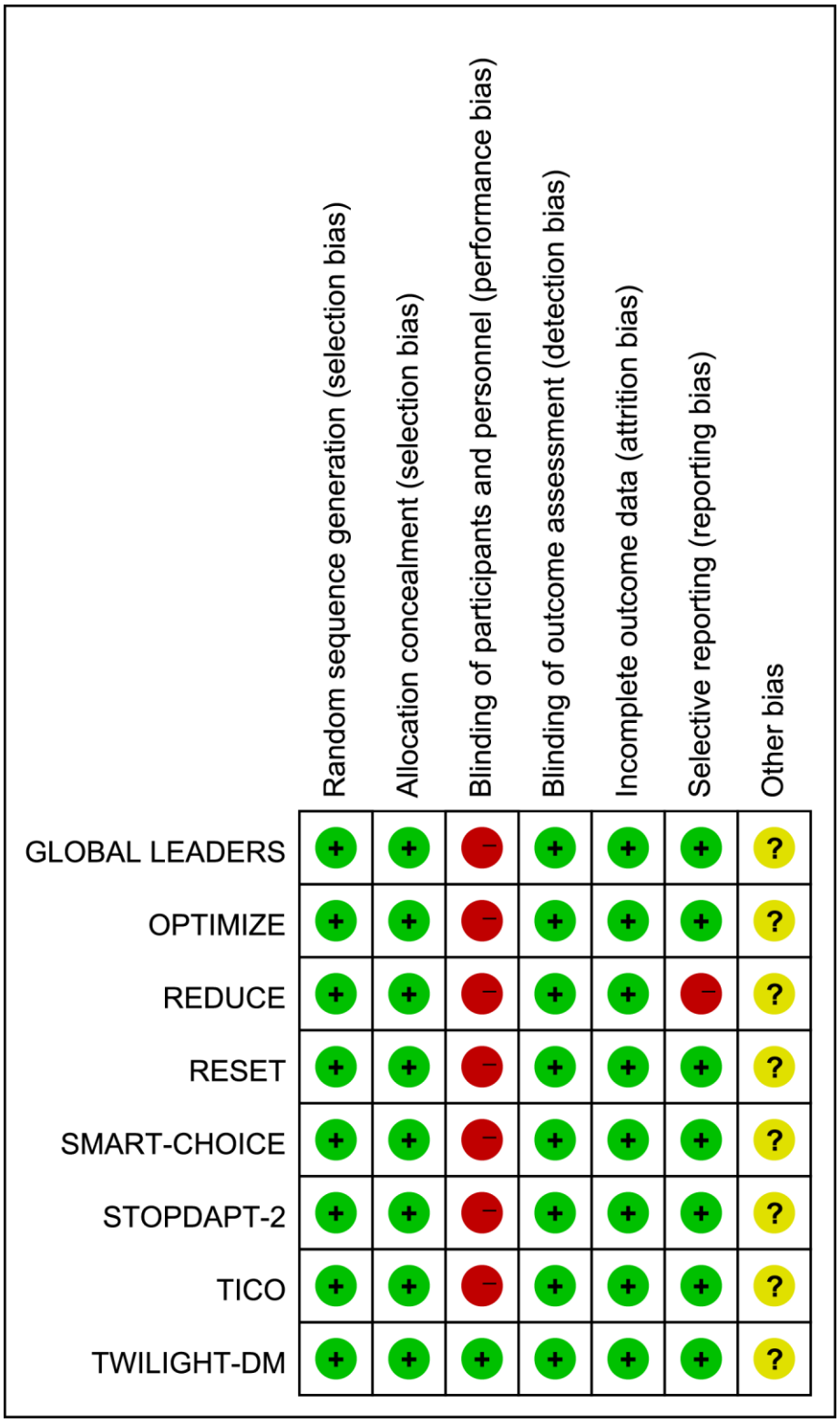
	aspirin	4/700	4/695	1.03 [0.27, 3.99]	0.96
stroke	$P2Y_{12}$	39/3357	38/3247	0.99 [0.63, 1.55]	0.97
	aspirin	1/554	2/549	0.50 [0.05, 5.45]	0.57
major bleeding	$P2Y_{12}$	78/3786	105/3707	0.64 [0.31, 1.32]	0.23
	aspirin	1/554	0/549	2.97 [0.12, 72.87]	0.5
Non- diabetes mellitus					
NACE	$P2Y_{12}$	143/4260	167/4310	0.77 [0.49, 1.22]	0.27
	aspirin	139/2592	140/2608	1.00 [0.80, 1.26]	0.99
all-cause death	$P2Y_{12}$	162/8141	177/8224	0.93 [0.75, 1.14]	0.47
	aspirin	22/1009	26/1007	0.84 [0.48, 1.48]	0.55
cardiac death	$P2Y_{12}$	75/8141	77/8224	0.91 [0.53, 1.55]	0.72
	aspirin	15/1922	20/1919	0.76 [0.29, 1.48]	0.42
MI	$P2Y_{12}$	213/8141	211/8224	1.06 [0.80, 1.40]	0.71
	aspirin	34/1922	27/1919	1.26 [0.76, 2.08]	0.37
Probable or definite ST	$P2Y_{12}$	70/9056	70/9159	0.99 [0.71,1.38]	0.96
	aspirin	5/1922	3/1919	1.60 [0.36, 7.02]	0.53
stroke	$P2Y_{12}$	65/8141	73/8224	1.24 [0.41, 3.74]	0.70
	aspirin	4/1009	3/1007	1.33 [0.30, 5.93]	0.71
major bleeding	$P2Y_{12}$	144/9270	178/9369	0.75 [0.52, 1.07]	0.11
	aspirin	4/1009	6/1007	0.67 [0.19, 2.35]	0.53

Abbreviations: NACE=net adverse clinical event, MI=myocardial infarction, ST= stent thrombosis.

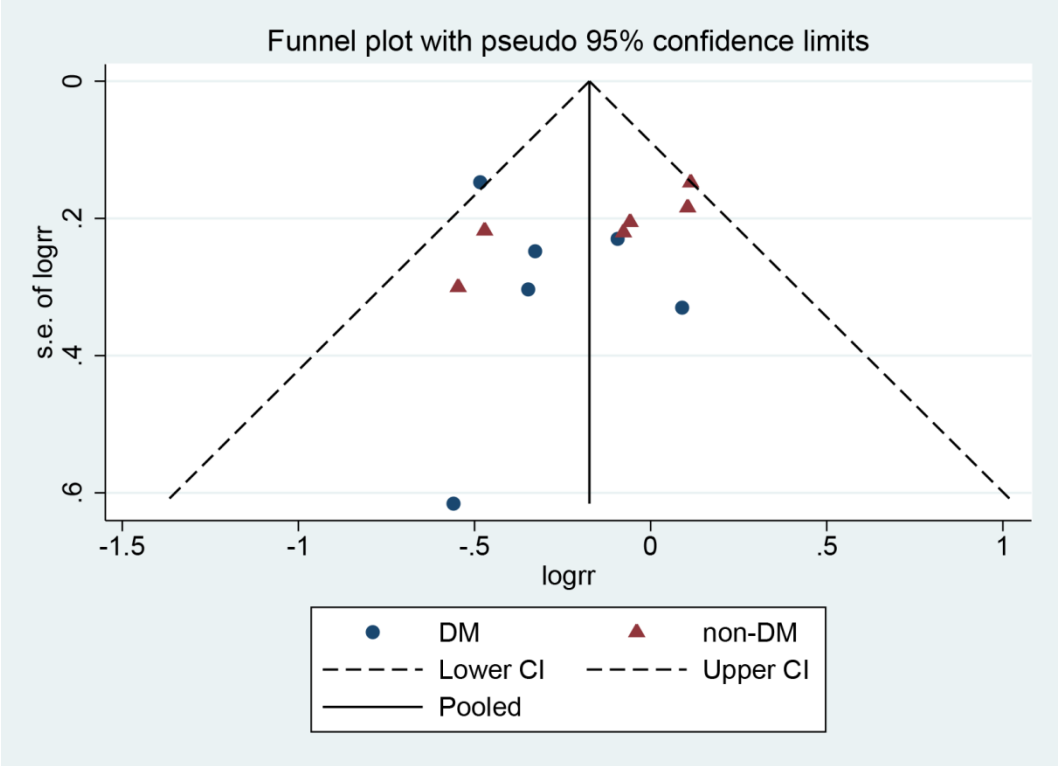
Supplementary Table 8. Subgroup analysis of different $P2Y_{12}$ antagonists in the very short term DAPT.

Total		1-3 months	12 months	RR [95% CI]	*P*
NACE	Ticagrelor	222/5081	285/5093	0.77 [0.62, 0.95]	0.02
	Clopidogrel	35/1500	55/1509	0.64 [0.42, 0.97]	0.04
Probable or definite ST	Ticagrelor	96/11498	101/11471	0.95 [0.72, 1.26]	0.72
	Clopidogrel	4/1500	1/1509	0.96 [0.67, 1.38]	0.21
Diabetes mellitus					
NACE	Ticagrelor	96/1727	148/1718	0.64 [0.50, 0.82]	0.0005
	Clopidogrel	18/585	25/574	0.71 [0.39, 1.28]	0.25
Probable or definite ST	Ticagrelor	29/3357	31/3247	0.91 [0.55, 1.51]	0.71
	Clopidogrel	1/585	1/574	0.98 [0.06, 15.65]	0.99
Non- diabetes mellitus					
NACE	Ticagrelor	126/3345	137/3375	0.85 [0.48, 1.51]	0.59
	Clopidogrel	17/915	30/935	0.58 [0.32, 1.04]	0.07
Probable or definite ST	Ticagrelor	67/8141	70/8824	0.97 [0.69, 1.35]	0.85
	Clopidogrel	3/915	0/935	7.15 [0.37, 138.28]	0.19

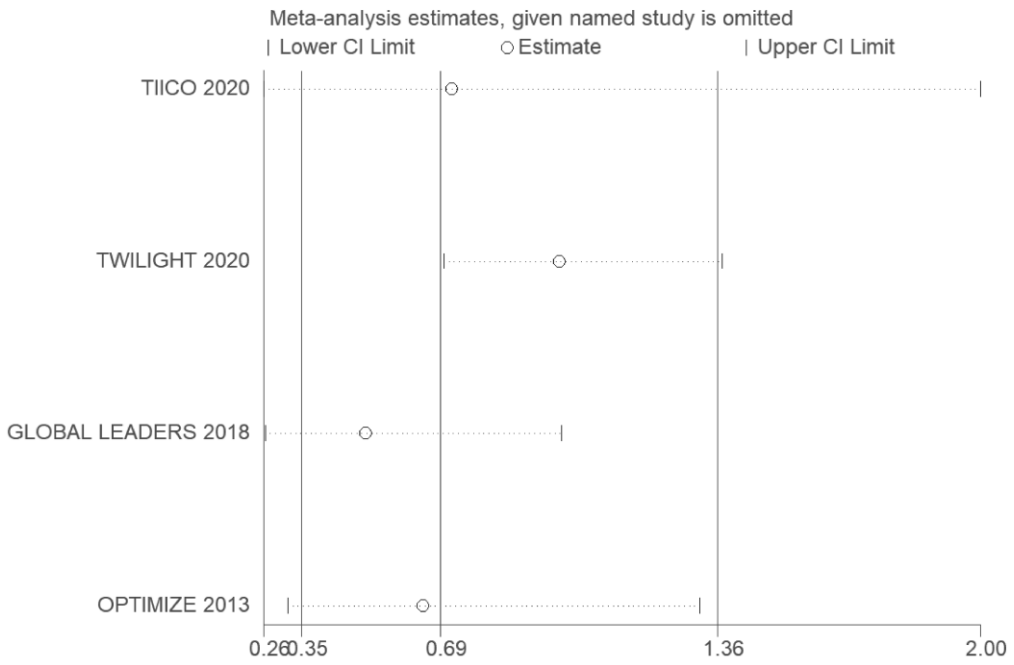
Abbreviations: NACE=net adverse clinical event, ST= stent thrombosis.



Supplementary Figure 1. Quality assessment of each randomized controlled trial included.



Supplementary Figure 2. The funnel plot of NACE outcome. (DM= diabetes mellitus, non-DM=non- diabetes mellitus.)



Supplementary Figure 3. Sensitivity analysis for outcome of major bleeding in patients with diabetes mellitus.